REFERAL LABS
Revolutionizing Healthcare

NABL M(EL)T 00417

nanocard Unlimited Testing

UNLIMITED TESTS UNLIMITED TIMES

Patient Name : MR. S V NANDARAJU
Age / Gender : 72 years / Male
Mobile No. : 9945211059
Patient ID : 34595
Source : HOME COLLECTION

Referral : SELF
Collection Time : Oct 21, 2024, 11:23 a.m.
Reporting Time : Oct 21, 2024, 02:14 p.m.
Sample ID : 000829524

Test Description	Value(s)	Unit(s)	Reference Range

COMPLETE BLOOD COUNT

Hemoglobin (Hb)* Method : Cynmeth Photometric Measurement	15.4	gm/dL	13.5-18.5
Erythrocyte (RBC) Count* Method : Electrical Impedence	5.05	mil/cu.mm	4.5 – 5.5
Packed Cell Volume (PCV)* Method : Calculated	46.2	%	40-54
Mean Cell Volume (MCV)* Method : Electrical Impedence	91.5	fL	76-100
Mean Cell Haemoglobin (MCH)* Method : Calculated	30.4	pg	25-32
Mean Corpuscular Hb Concn. (MCHC)* Method : Calculated	33.3	gm/dL	32-36
Total Leucocytes (WBC) Count* Method : Electrical Impedence	9500	cell/cu.mm	4500 – 11500
Neutrophils* Method : VCSn Technology	57.4	%	34 - 71
Lymphocytes* Method : VCSn Technology	34.1	%	20 – 45
Monocytes* Method : VCSn Technology	5.1	%	2 – 10
Eosinophils* Method : VCSn Technology	3.4	%	1 – 6
Basophils* Method : VCSn Technology	0	%	0 – 2
Platelet Count* Method : Electrical Impedence	255	10^3/ul	150-450

Test done on Automated five part cell Counter(WBC ,RBC,Platelet count by Impedence method , colorimetric method for Haemoglobin ,WBC differential by flow cytometry using laser technology
other parameters are calculated).

END OF REPORT

Saved By : CHANDINI

Result Submitted By : CHANDINI

Result Approved By : Shilpashree dixit

Patient Name : MR. S V NANDARAJU
Age / Gender : 72 years / Male
Mobile No. : 9945211059
Patient ID : 34595
Source : HOME COLLECTION

Referral : SELF
Collection Time : Oct 21, 2024, 11:23 a.m.
Reporting Time : Oct 21, 2024, 02:14 p.m.
Sample ID : 000829524

Test Description	Value(s)	Unit(s)	Reference Range

DR Shilpashree Dixit
KMC Registration 77264
Consultant Pathologist

Patient Name : MR. S V NANDARAJU
Age / Gender : 72 years / Male
Mobile No. : 9945211059
Patient ID : 34595
Source : HOME COLLECTION

Referral : SELF
Collection Time : Oct 21, 2024, 11:23 a.m.
Reporting Time : Oct 21, 2024, 03:14 p.m.
Sample ID : 000829524

Test Description	Value(s)	Unit(s)	Reference Range

Thyroid Function Test - Total

Thyroid Function Test

T3-Total **Method : CLIA**	1.196	ng/mL	0.8-1.9
T4-Total Method : CLIA	8.415	ng/dL	5.0-13.0
TSH-Thyroid Stimulating Hormone Method : CLIA	3.275	µIU/ml	0.35-4.75
Note :			

Saved By : Irshad A Result Submitted By : Irshad A Result Approved By : Jayashree

Vitamin D3 (25 - OH)

Vitamin D (25 - Hydroxy) **Method : CLIA**	20.391	ng/mL	Children aged 0-14 years: <12ng/mL as vitamin D deficiency, 12-20ng/mL as vitamin D insufficiency, 20-100ng/mL as vitamin D sufficiency, >100ng/mL as vitamin D overload. Adults: <20ng/mL as vitamin D deficiency; 20ng/mL~30ng/mL as vitamin D insufficiency; 30ng/mL~100ng/mL as vitamin D sufficiency; >100ng/mL as vitamin D overdose.
Note	source for the reference range is Endocrine Scociety		

Interpretation:

Useful for :
Diagnosis of vitamin D deficiency .
Differential diagnosis of causes of rickets and Osteomalacia . Monitoring vitamin D replacement therapy . Diagnosis of hypervitaminosis D .
Vitamin D levels may vary according to factors such as geography, season, or the patient's health, diet, age, ethnic origin, use of vitamin D supplementation or environment.
Some potential interfering substances like rheumatoid factor, endogenous alkaline phosphatase, fibrin, and proteins capable of binding to alkaline phosphatase in the patient sample may cause erroneous results in immunoassays.Carefully evaluate the results of patients suspected of having these types of interferences.

Saved By : Irshad A Result Submitted By : Irshad A Result Approved By : Jayashree

CRP-(C-Reactive Protein) Quantitative

CRP (C-REACTIVE PROTEIN)* Method : Immuno-turbidimetric	0.8	mg/L	0-6.0

Interpretation

1. Elevated test results can occur in patients with hypertension, elevated body mass index, metabolic syndrome/ diabetes mellitus, chronic infection (e.g., gingivitis, bronchitis), chronic inflammation (e.g. rheumatoid arthritis) and low HDL/high triglycerides
2. Cigarette smoking can cause increased levels.

3. Decreased test levels can results from moderate alcohol consumption, weight loss, and increased activity or endurance exercise

Saved By : Irshad A   Result Submitted By : Irshad A   Result Approved By : Jayashree

Renal Function Test (RFT)

Renal Function Test

Urea Method : Urease-GLDH	29.2	mg/dL	Adult Male <50 yrs : 19-44 ,>50 yrs : 18-55 Female <50 yrs : 15-40 ,>50 yrs : 21 - 43
Creatinine Method : ENZYMATIC	0.84	mg/dL	Children(1 yrs -14 yrs) : 0.30 - 0.70 Adult Male : 0.72 - 1.25 Adult Female : 0.57 - 1.11
Uric Acid **Method : Uricase**	5.9	mg/dL	3-5-7.0
Serum Calcium Method : ARSENAZO	9.1	mg/dl	8.4 - 10.2

Saved By : CHANDINI   Result Submitted By : CHANDINI   Result Approved By : Jayashree

LIVER FUNCTION TEST (LFT)

Liver Function Test

Bilirubin - Total Method : Diazonium Salt	0.80	mg/dL	Adults : 0.2 - 1.2
Bilirubin - Direct Method : Diazo Reaction	0.35	mg/dL	0.0 - 0.5
Bilirubin - Indirect Method : Serum, Calculated	0.45	mg/dL	0.1 - 1.0
SGOT (AST) Method : NADH without P-5-P	16.7	U/L	5 - 35
SGPT (ALT) Method : NADH without P-5-P	27.8	U/L	0 - 55
Alkaline Phosphatase-ALP	64	U/L	MALE: 1 - 12 Yrs-<500 12-15 Yrs <750 > 20 150 FEMALE: 1 - 12 Yrs <500 > 15 Yrs - 40-150
Total Protein Method : Biuret	6.98	g/dL	6.4 - 8.3
Albumin Method : BCG	4.27	g/dL	3.5 - 5.2

Patient Name : MR. S V NANDARAJU
Age / Gender : 72 years / Male
Mobile No. : 9945211059
Patient ID : 34595
Source : HOME COLLECTION

Referral : SELF
Collection Time : Oct 21, 2024, 11:23 a.m.
Reporting Time : Oct 21, 2024, 03:14 p.m.
Sample ID : 000829524

Test Description	**Value(s)**	**Unit(s)**	**Reference Range**
A/G Ratio Method : Calculated	1.58		1.0 - 1.8
Globulin Method : Calculated	2.71		2.3 - 3.5

Saved By : CHANDINI   Result Submitted By : CHANDINI   Result Approved By : Jayashree

Electrolytes

Electrolytes

Sodium Method : Indirect ISE	143.9	mmol/L	136 - 145
Potassium Method : Indirect ISE	4.20	mmol/L	3.5 - 5.1
Chloride Method : Indirect ISE	101.6	mmol/L	96 - 108

- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Diagnostic Lab issuing this report nor its employees/representatives assume any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report.

END OF REPORT

Saved By : Irshad A   Result Submitted By : Irshad A   Result Approved By : Jayashree

Dr. Jayashree
Ph.D. in Biochemistry

Patient Name : MR. S V NANDARAJU
Age / Gender : 72 years / Male
Mobile No. : 9945211059
Patient ID : 34595
Source : HOME COLLECTION

Referral : SELF
Collection Time : Oct 21, 2024, 11:23 a.m.
Reporting Time : Oct 21, 2024, 01:37 p.m.
Sample ID : 000829524

Test Description	Value(s)	Unit(s)	Reference Range
ERYTHROCYTE SEDIMENTATION RATE - ESR			
ESR - Erythrocyte Sedimentation Rate	8.17	mm/hr	0 - 10

Interpretation:

ESR is also known as Erythrocyte Sedimentation Rate. An ESR test is used to assess inflammation in the body. Many conditions can cause an abnormal ESR, so an ESR test is typically used with other tests to diagnose and monitor different diseases. An elevated ESR may occur in inflammatory conditions including infection, rheumatoid arthritis ,systemic vasculitis, anemia, multiple myeloma , etc. Low levels are typically seen in congestive heart failure, polycythemia ,sickle cell anemia, hypo fibrinogenemia , etc.

Reference- Dacie and lewis practical hematology

END OF REPORT

Saved By : CHANDINI

Result Submitted By : CHANDINI

Result Approved By : Shilpashree dixit

Dr.dixit

DR Shilpashree Dixit
KMC Registration 77264
Consultant Pathologist

Patient Name : MR. S V NANDARAJU
Age / Gender : 72 years / Male
Mobile No. : 9945211059
Patient ID : 34595
Source : HOME COLLECTION

Referral : SELF
Collection Time : Oct 21, 2024, 11:23 a.m.
Reporting Time : Oct 21, 2024, 04:08 p.m.
Sample ID : 000829524

Test Description	Value(s)	Unit(s)	Reference Range

Iron (Total)

Serum Iron Method : Ferrozine method without deproteinization	130.7	µg/dl	65 - 175

Bio. Ref. Interval. :
Males: 65 - 175
Females: 50 - 170

Saved By : Irshad A   Result Submitted By : Irshad A   Result Approved By : Jayashree

RANDOM BLOOD SUGAR

Glucose Random **Method : Plasma, Hexokinase**	139.7	mg/dL	Non Diabetic < 140 Mg /dl Pre Diabetes 141 -199 mg /dl Diabetes Mellitus 200 or higher mg /dl

Saved By : CHANDINI   Result Submitted By : CHANDINI   Result Approved By : Jayashree

Glycosylated Hemoglobin - HbA1c (H.P.L.C - METHOD)

HbA1c (Glycated Haemoglobin) , WHOLE BLOOD EDTA

HbA1c (Glycated Haemoglobin) **Method : HPLC**	**10.3**	%	<5.7
Estimated Average Glucose **Method : Calculated**	248.91	mg/dL	-

Patient Name : MR. S V NANDARAJU
Age / Gender : 72 years / Male
Mobile No. : 9945211059
Patient ID : 34595
Source : HOME COLLECTION

Referral : SELF
Collection Time : Oct 21, 2024, 11:23 a.m.
Reporting Time : Oct 21, 2024, 04:08 p.m.
Sample ID : 000829524

Test Description	**Value(s)**	**Unit(s)**	**Reference Range**

Interpretation:

Interpretation For HbA1c% As per American Diabetes Association (ADA)

Diagnosing Diabetes	
Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Age < 19 years Goal of therapy: <7.5

Note:

1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled.
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.

Comments : HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations ADA criteria for correlation between HbA1c & Mean plasma glucose levels.

HbA1c(%)	Mean Plasma Glucose (mg/dL)	HbA1c(%)	Mean Plasma Glucose (mg/dL)
6	126	12	298
8	183	14	355
10	240	16	413

Graph

Saved By : Irshad A   Result Submitted By : Irshad A   Result Approved By : Jayashree

Lipid Profile

Lipid Profile

Patient Name : MR. S V NANDARAJU
Age / Gender : 72 years / Male
Mobile No. : 9945211059
Patient ID : 34595
Source : HOME COLLECTION

Referral : SELF
Collection Time : Oct 21, 2024, 11:23 a.m.
Reporting Time : Oct 21, 2024, 04:08 p.m.
Sample ID : 000829524

Test Description	Value(s)	Unit(s)	Reference Range
Cholesterol-Total Method : CHOD-PAP	144	mg/dL	Desirable: <= 200 Borderline High: 201-239 High: >= 239
Triglycerides Method : Glycerol Oxidase - Peroxidase	140.2	mg/dL	Normal: < 150 Borderline High: 150-199 High: 200-499 Very High: >= 500
Cholesterol-HDL Direct Method : PVS-PEGME	**31.0**	mg/dL	35.3-79.5
LDL Cholesterol Method : Calculated	84.96	mg/dL	Optimal: < 100 Near optimal/above optimal: 100-129 Borderline high: 130-159 High: 160-189 Very High: >= 190
Non - HDL Cholesterol, Serum Method : calculated	113	mg/dL	Desirable: < 130 Borderline High: 130-159 High: 160-189 Very High: > or = 190
VLDL Cholesterol Method : calculated	28.04	mg/dL	6 - 38

Saved By : CHANDINI Result Submitted By : CHANDINI Result Approved By : Jayashree

Vitamin B12

Vitamin B12 (Cyanocobalamin) **Method : CLIA**	222.024	pg/mL	183-822

Note

source for the reference range is Kit Insert

Interpretation:

Vitamin B12, also known as cyanocobalamin, is a water soluble vitamin that is required for the maturation of erythrocytes and coenzyme form for more than 12 different enzyme systems. Groupsat risk for vitamin B12 deficiency include those

(1) older than 65 years of age (2) with malabsorption(3) who are vegetarians (4) with autoimmune disorders(5) taking prescribedmedication known to interfere with vitamin absorption or metabolism, including nitrous oxide, phenytoin, dihydrofolate reductase inhibitors, metformin, and proton pump inhibitors(6) infants with suspected metabolic disorders.

The most common cause of Vitamin B12 deficiency is pernicious anemia. Deficiency of Vitamin B12 is associated with megaloblastic anemia and neuropathy. Excess Vitamin B12 is excreted in urine. No adverse effects have been associated with excess vitamin B12 intake from food or supplements in healthy people.

Patient Name : MR. S V NANDARAJU
Age / Gender : 72 years / Male
Mobile No. : 9945211059
Patient ID : 34595
Source : HOME COLLECTION

Referral : SELF
Collection Time : Oct 21, 2024, 11:23 a.m.
Reporting Time : Oct 21, 2024, 04:08 p.m.
Sample ID : 000829524

Test Description	Value(s)	Unit(s)	Reference Range

END OF REPORT

Saved By : Irshad A

Result Submitted By : Irshad A

Result Approved By : Jayashree

Dr. Jayashree
Ph.D. in Biochemistry